

DATAGOV TOOLS V2 · SAMPLE TEST PROJECT · FILLING GUIDE

Health Claims Analytics

Expected Project ID PRJ-2026-024 · PT Asuransi Sehat Bersama (Dummy) · all names, IDs and numbers are fictitious

Sample project for a health insurer that analyses claims to forecast costs and detect unusual claims. It tests health data (specific personal data under UU PDP), two DSRs (one with AI use and its AICK, one without), a DPIA with High residual risk, and Data Quality with duplicated claim IDs, gaps that are expected by design, and a sensitive column that should not be there.

Files (folder `Dummy Data Source/PRJ-2026-024/`)

File (sheet)	Rows x columns	Used for
HCL01_Members.xlsx (members)	250 x 11	Metadata (personal + religion), DQ
HCL02_Claims.xlsx (claims)	800 x 9	Metadata (health data), DQ, DSR 1 and DSR 2
HCL03_Providers.xlsx (providers)	60 x 7	Metadata (reference table), DQ

Field names and buttons follow the current version of the application. The Project ID is assigned by the system (the next free PRJ-2026-NNN); PRJ-2026-024 assumes the three sample projects are created in order after PRJ-2026-022.

Step 1: Create the project (Data Assets Catalog > New Data Asset)

Project ID / Code	assigned by the system (expected PRJ-2026-024)
Project / Asset Name	Health Claims Analytics
Customer / Client Name	PT Asuransi Sehat Bersama (Dummy)
Line of Business	Health Insurance
Project Category	Analytics
Project Year	2026
Monetization Status	Internal Governance
Start Date	2026-11-01
End Date	2027-10-31
Use Case & Governance Description	Analyse health insurance claims to forecast claim costs per plan and flag unusual claims for review. Data: member master, claims from June to September 2026 and the provider list. Health and identity data are pseudonymised before analysis; diagnosis codes are used at ICD-10 level only; access is limited to the project team.

Delivery & Governance Team (pick the existing users)

Role	User
Subject Matter Expert (SME)	Dewi Rahayu (Head of Business Analytics)
Delivery Manager	Shinta Kusumawati (Senior Project Manager)
Project Manager	Bagas Adi Nugraha (Lead Project Manager)
Data Governance Officer (DGO)	Hendra Wijaya (Data Governance Analyst)
Metadata Officer	Rahmat Hidayat (Metadata Analyst)
Data Quality Officer (DQO)	Desi Wulandari (Data Quality Analyst)
PIC Data Compliance	Ayu Lestari (Compliance Officer)

Accountable Stewardship & Ownership

Role	Full Name	Position	Email Address
Lead Business Steward	Bayu Anggara	-	bayu.anggara@asuransisehat.example
Data Owner	dr. Ratih Kusumawardani	Head of Claims Management	ratih.kusumawardani@asuransisehat.example

Check: the project page shows **Uploaded Source Files Kept Until 30 Nov 2027** (end date + 30 days, no approved ROPA).

Step 2: Metadata (Metadata Management > select the project > import files)

- 1. Select the project, Data Source Connector **Excel**, upload HCL01, HCL02 and HCL03, then **Discover Source Tables**.
- 2. Select the 3 tables and click **Proceed Metadata (3)**: expect `members` (11 attributes), `claims` (9) and `providers` (7) = 27 attributes.
- 3. Open the attributes grid and click **Generate AI Definitions**.
- 4. Set **Table Type** of `providers` to **Reference** and its **Level** to Raw (it is a reference list).

Check: `full_name`, `nik`, `birth_date`, `gender`, `email`, `phone` and `religion` are **Highly Confidential**. Review point: `diagnosis_code` is health data but its name matches no keyword, so it is not flagged; change its Sensitivity to Highly Confidential by hand and Save.

Step 3: DSR 1, AI use (Data Sharing (DSR) > New Request)

Source Project	Health Claims Analytics
Dataset Name	Claims Cost Features (Pseudonymised)
Recipient Organisation	PT Aktuaria Prima Analitika (Dummy)

Purpose & Legal Justification	Share pseudonymised claim records (HCL02, member_id hashed, no names) with the actuarial analytics vendor to build a claims-cost forecasting and anomaly model. Legal basis: contract (policy terms on claims administration) and legitimate interest in fraud prevention. Controls: ICD-10 codes only, no free-text medical notes, 2 named vendor analysts inside the insurer's analytics workspace, data deleted at project end.
Sharing Start Date	2026-11-10
Sharing End Date	2027-10-31
This dataset will be consumed for AI / Machine Learning workflows	Ticked

Check: an AICK (expected AICK-2026-0009) and the project's DPIA draft are created automatically.

Data & Insights Sharing Evaluation Checklist (on the DSR page, Edit)

Every item needs Yes/No and a remark. For A and C, "No" is the good answer.

#	Question	Answer	Remarks
A.i.1	Loss in revenue due to cannibalization	No	The data is used only for the insurer's own claims analysis; the recipient may not reuse or resell it.
A.i.2	Damage to relationship with customers	No	Members are not contacted by the recipient; member identities are not shared.
A.i.3	Other commercial interest(s)	No	No premium pricing or provider tariff agreements in the dataset.
A.ii.1	Highly confidential partnerships	No	No confidential provider partnership terms included.
A.ii.2	Patent information	No	No patent information.
A.ii.3	M&A deals	No	Not related to any merger or acquisition.
A.ii.4	Other commercial secret(s)	No	Claim reserving methods and reinsurance pricing are not included.
B.i	Are the data & insights requested sensitive?	Yes	Diagnosis codes and claim amounts are health data (specific personal data under UU PDP).
B.ii	Are there any mitigation steps in place if sensitive data & insights are used?	Yes	member_id hashed; name, NIK, e-mail, phone and birth date removed; diagnosis kept at ICD-10 code level only.
B.iii	Are the data & insights requested considered personal data?	Yes	Pseudonymised claims remain personal data because the insurer keeps the key table.
B.iv	If personal data need to be shared, has written consent for sharing been obtained?	Yes	Policy terms clause 9 allow processing by appointed processors; the vendor signed DPA-2026-033 (dummy).
B.v	If personal data need to be used for use case development, has written consent for research been obtained?	Yes	Policy terms cover analysis for claims management and fraud prevention; lapsed members are excluded.
B.vi	Are the data and analytics processes located within the BU's analytics environment with limited access to the working team?	Yes	Vendor works inside the insurer's analytics workspace, 2 named accounts, no download.

C.i.1	Industry-specific laws	No	Insurance regulation (OJK) allows sharing with a contracted processor and with reinsurers under a treaty.
C.i.2	Data protection laws	No	UU PDP: health data is specific personal data; legal basis, processor agreement, pseudonymisation and DPIA in place.
C.i.3	Internal regulations and policies	No	Follows the Group Data Sharing Policy: DSR approval, DPIA, RBAC and deletion at the end of the use.
D.i	Is the data analysis process carried out using artificial intelligence technology?	Yes	The vendor trains a forecasting and anomaly model; the AICK is completed and flagged claims are always reviewed by a claims officer.

Sign-off

Approved?	Yes
Prepared By	dr. Ratih Kusumawardani / Head of Claims Management (auto-filled from the project Data Owner)
Acknowledged By	Dewi Rahayu / Head of Business Analytics (auto-filled from the project SME)
Remarks	Approved with conditions: (1) pseudonymised claims only, no identity data and no medical notes; (2) 2 named vendor accounts inside the analytics workspace; (3) the model only flags claims for human review and never rejects a claim by itself; (4) data deleted at project end. Ref: DPA-2026-033 (dummy).

Approval flow: PIC Data Compliance Approval (Ayu Lestari) → DM / PM Approval → SME Sign Off → Client Sign Off.

Step 3b: DSR 2, no AI use (Data Sharing (DSR) > New Request)

Source Project	Health Claims Analytics
Dataset Name	Approved Claims Q3 2026
Recipient Organisation	PT Reasuransi Mitra Utama (Dummy)
Purpose & Legal Justification	Provide approved claims of Q3 2026 (claim amount, approved amount, diagnosis code, claim date) to the reinsurer for the quarterly treaty settlement. Legal basis: contract (reinsurance treaty RI-2026-04, dummy). Member IDs are hashed; no names or contact details.
Sharing Start Date	2026-11-15
Sharing End Date	2026-12-31
This dataset will be consumed for AI / Machine Learning workflows	Not ticked

Check: no AICK is created for this DSR (AI box not ticked), and no second DPIA (the project already has one).

Data & Insights Sharing Evaluation Checklist (on the DSR page, Edit)

Every item needs Yes/No and a remark. For A and C, "No" is the good answer. Section D does not appear (no AI use).

#	Question	Answer	Remarks
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A.i.1	Loss in revenue due to cannibalization	No	The data is used only for the insurer's own claims analysis; the recipient may not reuse or resell it.
A.i.2	Damage to relationship with customers	No	Members are not contacted by the recipient; member identities are not shared.
A.i.3	Other commercial interest(s)	No	No premium pricing or provider tariff agreements in the dataset.
A.ii.1	Highly confidential partnerships	No	No confidential provider partnership terms included.
A.ii.2	Patent information	No	No patent information.
A.ii.3	M&A deals	No	Not related to any merger or acquisition.
A.ii.4	Other commercial secret(s)	No	Claim reserving methods and reinsurance pricing are not included.
B.i	Are the data & insights requested sensitive?	Yes	Diagnosis codes are health data.
B.ii	Are there any mitigation steps in place if sensitive data & insights are used?	Yes	Hashed member_id only; no identity or contact data.
B.iii	Are the data & insights requested considered personal data?	Yes	Linkable to members through the insurer's key table, so personal data.
B.iv	If personal data need to be shared, has written consent for sharing been obtained?	Yes	Sharing with reinsurers is stated in the policy terms (clause 14) and required by the treaty.
B.v	If personal data need to be used for use case development, has written consent for research been obtained?	No	Not used for use case development, only for the treaty settlement.
B.vi	Are the data and analytics processes located within the BU's analytics environment with limited access to the working team?	Yes	Delivered through the insurer's secure file exchange; 2 named reinsurer staff.
C.i.1	Industry-specific laws	No	Insurance regulation (OJK) allows sharing with a contracted processor and with reinsurers under a treaty.
C.i.2	Data protection laws	No	UU PDP: health data is specific personal data; legal basis, processor agreement, pseudonymisation and DPIA in place.
C.i.3	Internal regulations and policies	No	Follows the Group Data Sharing Policy: DSR approval, DPIA, RBAC and deletion at the end of the use.

Sign-off

Approved?	Yes
Prepared By	dr. Ratih Kusumawardani / Head of Claims Management (auto-filled)
Acknowledged By	Dewi Rahayu / Head of Business Analytics (auto-filled)
Remarks	Approved for the Q3 2026 treaty settlement only: approved claims with hashed member_id, secure file exchange, access ends 2026-12-31; the reinsurer confirms deletion after settlement.

Approval flow: PIC Data Compliance Approval (Ayu Lestari) → DM / PM Approval → SME Sign Off → Client Sign Off.

Step 4: AI Compliance Checklist (AI/ML Checklist > AICK-2026-0009 (expected))

Gen AI tools in this project: the platform's local llama3.2 (on-premise Ollama) drafts Metadata definitions and DQ rules; the vendor uses an approved enterprise chat assistant to draft analysis reports. No medical data goes into any prompt.

Created automatically with the AI-use DSR. Open it from **AI/ML Checklist**, click Edit, fill in Status and Remarks for each item, then the sign-off, and Submit.

Gen AI Usage Assessment

Area	Risk	Assessment	Status	Remarks
Before Use	HIGH	The project or product developed using an AI platform has complied with applicable legal regulations and internal company policies	Yes	Use case reviewed against UU PDP (specific personal data), OJK insurance rules and the Group AI Guideline; DPIA completed.
Before Use	HIGH	The Gen AI Platform used is properly licensed and has been approved by management	Yes	llama3.2 runs on-premise (approved by IT); the vendor's chat assistant is an approved enterprise licence.
Before Use	MEDIUM	Gen-AI usage settings have been configured to disable interaction history and ensure opt-out from model training by default	Yes	History disabled and training opt-out set at organisation level for the chat assistant; llama3.2 keeps no history.
Input	HIGH	Do not include personal, sensitive, or intellectual property data.	Yes	Prompts never contain diagnosis codes, member data or claim details; only column names and masked samples.
Input	HIGH	Interactions with Gen-AI should utilize anonymization, pseudonymization, and dummy data.	Yes	Exploration uses pseudonymised data and the dummy files HCL01-HCL03.
Input	LOW	Prompts must not contain bias or harmful narratives, and should be documented if used for reporting or publication purposes.	Yes	Prompts used for reports are saved in the project folder (prompt-log.txt) and reviewed by the DGO.
Output	MEDIUM	Interactions with Gen-AI must include verification and revalidation of the results to ensure the validity of the information, including inaccuracies, hallucinations, and bias	Yes	AI-drafted definitions, DQ rules and report text are checked by the Metadata Officer, DQ Officer and SME.
Output	HIGH	Any source code generated by Gen-AI must be properly validated before being implemented in the Company's systems	Yes	No AI-generated code goes into production systems without review and tests.
Utilization	MEDIUM	Outputs should only be used after being reviewed and approved by an authorized user.	Yes	Model results are used only after review by the Head of Claims Management; flagged claims are reviewed by a claims officer.
Utilization	HIGH	Corrections must be made if the AI-generated results are inaccurate or inappropriate before any distribution.	Yes	Incorrect AI text is corrected before any report is distributed.

Sign-off (auto-filled from the project)

Approved?	Yes
Prepared By — Delivery Manager	Shinta Kusumawati / Senior Project Manager (Delivery Manager)
Acknowledged By — Subject Matter Expert (SME)	Dewi Rahayu / Head of Business Analytics
Remarks	Approved: Gen AI only for documentation, DQ rules and report drafting; no health or identity data in prompts.

Approval flow: PIC Data Compliance Approval (Ayu Lestari) → DM Sign-off → SME Sign-off. It shows **Completed & Signed** only after both signatures or the last approval.

Negative test (optional): set item Input 1 to No with the remark "Diagnosis descriptions were pasted into the chat assistant" and check the reviewers' reaction.

Step 5: DPIA (open the draft from the DPIA list, click Edit)

The DPIA draft was created automatically with the project's first DSR (one DPIA per project, status Draft). Do not use DPIA > New: open the draft from the DPIA list, click Edit, fill in the fields below and submit.

Process name	Health claims cost forecasting and anomaly review
Risk Description	Claims combine identity and health data (diagnosis codes). Risks: exposure of health data if the files leak, members treated unfairly if flagged claims are rejected automatically, and use of religion data that is not needed.
Mitigation Measures	Pseudonymise member data; ICD-10 codes only, no medical notes; human review of every flagged claim; remove the religion column from all extracts; access for named staff only; data deleted at project end.
Residual Risk Level	High

Data Categories Involved (tick these)

- **Personal Identity:** Full Name, National ID / IC Number, Date of Birth, Gender
- **Contact Information:** Email Address, Phone Number
- **Health & Medical:** Health Status & Diagnosis, Medical Insurance Claims
- **Financial:** Insurance Data
- **Special Category (Sensitive):** Religious / Philosophical Beliefs

Governance Activities

Status: Yes = in place or planned; No = not applicable (the Responsible then shows N/A).

#	Activity	Responsible	Status	Remarks
A1	Data Sharing Request Document	ADI-DI (Internal)	Yes	DSR 1 (vendor, AI use) and DSR 2 (reinsurer) submitted in the platform.
A2	Revoke / Extermination Documentation	ADI-DI (Internal)	Yes	Deletion records planned: vendor at project end, reinsurer after the Q3 settlement.
A3	Data Activity Records Documentation (during project)	ADI-DI (Internal)	Yes	All access logged in the audit log; monthly access review by the DGO (Hendra Wijaya).

A4	Role-Based Access Control Document	ADI-DI (Internal)	Yes	RBAC matrix v1.0: 2 vendor analysts, 2 reinsurer staff, project team per role.
A5	Assess and Approve Documentation Above	Client	Yes	Reviewed and approved by dr. Ratih Kusumawardani (Head of Claims Management).
A6	Grant Access to Project Team Only (per RBAC document)	Client	Yes	Access granted by the insurer's IT team per RBAC v1.0.
B1	Prepare Mitigated Personal Data (e.g. encrypted, hashed, pseudonymised)	Client	Yes	member_id hashed; identity columns and religion removed before any sharing.
C1	Provide Secure Environment or Schema to Enable Data Sharing	Client	Yes	Analytics workspace with MFA and no download; secure file exchange for the reinsurer.
D1	Create Metadata Documentation	ADI-DI (Internal)	Yes	Metadata for 3 tables (27 attributes); diagnosis_code set to Highly Confidential by hand.
D2	Measure Data Quality Index	ADI-DI (Internal)	Yes	DQ run on all 3 files; duplicated claim IDs and the religion column flagged for follow-up.
D3	Assess and Approve Metadata Definition	Client	Yes	Definitions reviewed by Bayu Anggara (Lead Business Steward).
D4	Assess and Approve Data Quality Measurement Approach and Index	Client	Yes	DQ approach and index reviewed with the Data Owner.

Approval flow: PIC Data Compliance → DM / PM Approval.

Negative test (optional): set the Residual Risk Level to Low and check whether reviewers challenge it (health data normally means High).

Step 6: Data Quality (Data Quality Control > select the project > Run All Data)

1. Data Quality Control: select the project, **Run All Data**; the 3 files are selected.
2. Generate: about 1 minute per column (11 + 9 + 7 = about 27 minutes). You can leave the page and come back through **View progress**.
3. When all 3 finish, open each run, then **Project Report**.

Expected results (facts from the data; exact regexes and rule text come from the AI model)

File	What to look for
HCL01 Members	Blank Attributes 0 / 11. religion Completeness 20.4% (Fail): the column is mostly empty and should not be collected at all (special category). Consistency: gender (8 values L/P instead of M/F), policy_no (5 without dashes, e.g. POL2025000123). Uniqueness for member_id, nik, policy_no, email, phone. Latency: birth_date and join_date 0% (latest join 2025-09-02).
HCL02 Claims	Blank Attributes 0 / 9. approved_amount_idr and paid_date Completeness 73.2% (Warning): empty by design for rejected and pending claims, a good review point. Consistency: diagnosis_code (12 like "j069" instead of J06.9), provider_code (6 like PRV0012 without the dash). No Uniqueness check for claim_id : 5 claim IDs are duplicated, and Uniqueness is created only for fully unique columns. Latency claim_date latest 2026-09-28 (70% within 7 days); paid_date up to 2026-10-14 (scheduled payments, 100%).

HCL03 Providers	Blank Attributes 1 / 7 (legacy_code). Small code sets provider_type (HOSPITAL/CLINIC/LAB) and bpjs_partner (Y/N). Uniqueness for provider_code . Latency contract_end 100% (future dates).
Project Report	Blank Attributes 1 / 27 ; all three tables in one report with table filter and export.

Discussion point: finding the duplicated claim IDs requires noticing that claim_id has no Uniqueness row; ask the tester to confirm it in the Rules tab.